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Long-Term Care System and Universal Health and Care Coverage

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KEY POINTS

- Developing member countries (DMCs) are aging rapidly and need Long-term care (LTC) systems that support person-centered, integrated care and aging in the right place, with appropriate and accessible care and support matched to older people's needs and preferences.
- The LTC system can improve efficiency by reducing social admissions in acute care hospitals and helping family caregivers remain or return to the labor force, thereby contributing to the care economy.
- Public financing for LTC needs a transparent system for assessing needs and eligibility that can prioritize those in greater care need.
- LTC policy priority will be higher for the training and certification of the formal care workforce and the service coverage of severe disability or residential care.
- DMCs should expand their systems to ensure that health care, LTC, and social support services are coordinated to meet the needs of rapidly aging populations through universal health and care coverage (UHCC).

WHAT IS LONG-TERM CARE?

Many Asian countries have been experiencing a rapid aging of population. Increase in life expectancy is often accompanied by the change in family structure, increased labor participation of women, and the change in attitude or cultural norms for family care-giving. Long-term care (LTC) is "a broad range of personal, social, and medical services and support for people with, or at risk of, significant loss of intrinsic capacity, aimed at maintaining functional ability consistent with dignity and basic rights" (WHO 2021a). LTC services can also provide assistance with basic activities of daily living (ADL), such as eating, dressing, bathing, using the toilet, as well as instrumental activities of daily living (IADL), such as cleaning, preparing meals, and shopping (OECD 2005).

Facing the increase in the demand for LTC, older people, especially from vulnerable groups, have limited capacity to pay for LTC as they do not have enough savings, or the public pension system is not well established in the majority of the countries. Policy for financing LTC affects the efficiency and equity of the (service) delivery of LTC as well as quality of life and financial protection for older people. LTC is provided in institutional

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settings, or more importantly, in the home or community environment; and in most countries, LTC programs are part of both the health and the social system (WHO 2021a).

Good LTC—which is person-centered, accessible, affordable, safe, and well-coordinated across health and social services—is essential for healthy aging, which is defined as "the process of developing and maintaining the functional ability that enables well-being in older age" (WHO 2021b). Functional ability is about having the capabilities that enable all people to be and do what they have reason to value, which includes a person's ability to meet their basic needs; learn, grow, and make decisions; be mobile; build and maintain relationships; and contribute to society (WHO 2021b).[1] The United Nations (UN) Decade of Healthy Ageing (2021–2030) also emphasizes LTC as a key component of healthy aging, i.e., "universal access to long-term care for older individuals with cognitive or physical dependency, including support for family caregivers, capacity-building for care workers, and the promotion of public–private partnerships" as well as a comprehensive, person-centered integrated system of care and health services.

WHY DO WE NEED A LONG-TERM CARE SYSTEM?

Currently, there is a lack of LTC systems in many countries in the region, and population aging and social changes call for building comprehensive LTC systems and governance. Life expectancy has increased, but healthy life expectancy has been stagnant in most countries (Choi et al. 2024). In other words, health and functional capacity of older people have not improved as quickly as life expectancy. Financial capacity (capacity to pay) of older people to pay for LTC is limited as pension is not sufficient. However, the potential of family caregiving has decreased due to urbanization, change in family structure, increase in labor participation of informal caregivers, and more importantly, the change in the attitude toward family caregiving for older people, i.e., a decreasing proportion of adults are willing to provide care for their older parents. Even when informal care or family caregiving is available, the quality of care varies and is often questionable because there is no formal system of quality assessment and improvement, in which case, the well-being of both caretakers and caregivers can deteriorate.

The burden of family care reduces labor participation of caregivers, who, in many countries, are women. When the caregivers for older people also have burdens of care for children, they suffer from the double burden of care, resulting in exit from the labor market or decrease in wage (Kim 2026). The care economy encompasses care work—paid and unpaid, direct and indirect—delivered through the public and private sectors (International Labour Organization. Care Economy topic portal); and a good LTC system and socialization of care increases the labor participation, contributing to the care economy. Unpaid care work (UCW) is a large part of the economy, and older women provide much of the care consumed by older people (Donehower 2025). When a country does not have an LTC system, older people tend to stay longer in acute care hospitals even when there is no need for medical care because LTC is not available or affordable after discharge. Therefore, building an LTC system can reduce the wasteful social admissions in acute care hospitals, resulting in efficiency in service delivery and the coordination of health care and LTC. Good LTC is also cost-effective and reduces the need for health care of older people when quality LTC improves disease prevention, health promotion, and health outcomes. This positive cycle improves the health and functional status of older people, as well as lessens health and LTC expenditure.

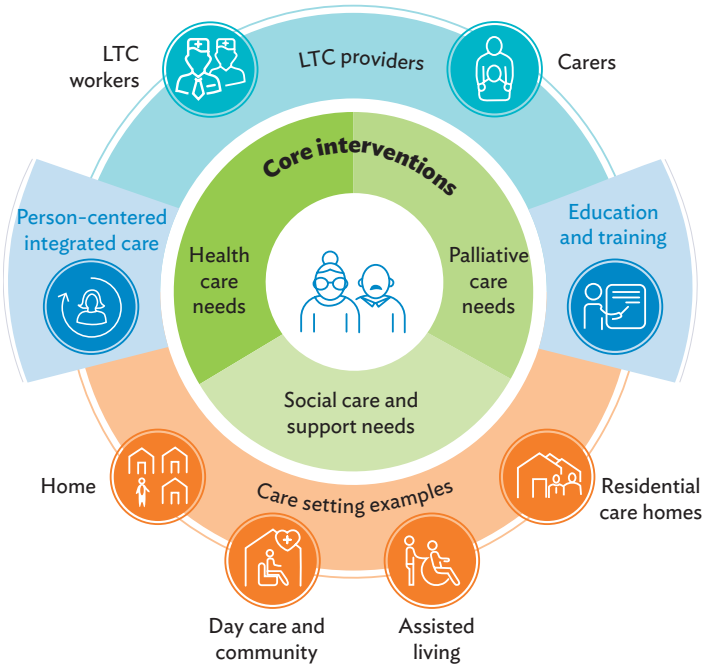
Beyond improving efficiency, the LTC system improves the quality of life of older people and their families. The unmet need for LTC and its hidden social cost tend to be underestimated in many Asian countries (Rahman et al. 2022). The LTC system can improve equity in the access to LTC, reduce unmet care needs, and improve financial protection, e.g., reducing catastrophic expenditure. A formal LTC system with certified care workers can improve the quality of LTC, thanks to a formal system of quality review and assessment, financial incentives, regulation of service provision, and others.

LONG-TERM CARE SERVICE DELIVERY AND WORKFORCE

Coverage and delivery of LTC. LTC can be provided in various modalities, such as home-based care, residential or institutional care, with a continuum of different types of care, e.g., daycare centers; cohousing, coliving, or group home; and respite care. Integrated care or continuum of care (prevention or promotion, acute care, long-term care, and social welfare services) has to be built; and it should be ensured that older people can choose care based on needs and preferences, and without financial barriers. The policy goal is to encourage person-centered integrated care and aging in the right place for living in a setting that best matches an older person's needs, preferences, vulnerabilities, and available supports, rather than remaining in the same home regardless of fit. The aim should be to avoid overreliance on institutional or residential care, rather than to eliminate it, because some older people prefer residential care or cannot stay home due to the high severity of functional status or unavailability of family caregivers. The absence of good residential LTC may result in over-medicalization and wasteful social admissions in hospitals. The following figure shows the key elements of coverage, service package, and workforce of an LTC system.

[1] Other components are *combating ageism* to eliminate age-based stereotypes and discrimination while fostering intergenerational respect and inclusion; *creating age-friendly environments* to transform both physical and social environments—including urban planning, transportation, housing, and access to information.

Groups of Core Long-Term Care Interventions and Facilitating Factors Provided by Carers in Various Care Settings



LTC = long-term care.
Source: World Health Organization. 2024. *Long-term care for older people: package for universal health coverage*. Geneva. p. xii.

In addition to supporting the activities of daily living (for those who have several functional limitations) as the core list of services, more new services to support "healthy aging" are being developed, e.g., social services, transportation, and social participation support, based on the local contexts. Ideally, the aim should be to ensure health and care across the life course and across sectors, coordinating health, social welfare, housing, labor, transportation, education, and others. Digital technology can be an effective tool to promote person-centered coordination across sectors, better responding to the need of older people. For example, remote monitoring, smart-home sensors, and artificial intelligence (AI)-driven systems can enable earlier detection of health deterioration, fall and emergency alerts, and continuous monitoring of daily activities, with evidence that home telemonitoring can significantly reduce hospital readmissions and help prevent medical emergencies (Sapci and Sapci 2019).

Workforce. As LTC is a mainly personal service, the LTC workforce is a key element of the LTC system. A formal LTC system cannot be sustainable and cannot contribute to the care economy without formal care workers, although family caregivers or voluntary workers can still play an important role. The LTC system needs training and certification for formal care workers, which highly depends on country contexts, e.g., the requirement and length of training, types of certificates, and career path. Government needs a policy on human resource competence and requirement for different types of LTC providers, such as visiting care providers and residential care facilities. The LTC system needs to be based on multidisciplinary teams of health and care professionals which provide coordinated care in the right settings. Many high-income economies suffer from the shortage of the care workforce due to low pay, heavy workloads, and high turnover; and importing of foreign workers has been actively pursued or discussed, e.g., in Hong Kong, China; Japan; and the Republic of Korea.

In the LTC system, a financing mechanism as a purchaser plays a key role in quality of care. Quality of LTC can be measured in terms of outcomes (e.g., satisfaction and improvement in functional status) as well as process (e.g., care planning and communication, food, nursing, medication, referral to medical providers). The purchaser defines the standard benefits and service package, (minimum) capacity of care workers, quality of care, and others. It can also review and assess the quality of LTC, provide financial and nonfinancial incentives, and accredit service providers.

FINANCING LONG-TERM CARE

Financing Models for Long-Term Care

Public financing, based either on tax or budget, or on mandatory social insurance, is key to reducing financial barriers to LTC and improving financial protection associated with the cost of

LTC. The role and type of public financing for LTC in a given country depends on the extent of population aging and old-age dependency, health and disability of older people (i.e., need for LTC), and welfare state orientation. Different LTC financing systems have different levels of eligibility and needs assessment, which defines population coverage, and various generosity of the benefits coverage and the cost-sharing scheme (copayment).

Public financing is preferred because people face access barriers in case of private financing, such as out-of-pocket payment. Private insurance exists in some high-income countries to complement public coverage, but voluntary private insurance markets tend to remain limited and do not provide equitable coverage nor sufficient protection against LTC costs. In principle, public financing is more efficient and equitable than private financing for LTC because public financing provides pooling and redistribution across both risk (functional capacity) and the capacity to pay while out-of-pocket pay has no pooling function. For example, contribution is related to ability to pay, but service utilization is based on the need for LTC in the public financing mechanism, resulting in benefiting functionally disabled and poor people.

Population Coverage

In principle, public financing for LTC covers those who have need for LTC. In practice, it can cover or target those in greater need, e.g., high severity or dependency and those living alone. Most public financing for LTC has a formal system for need assessment and eligibility. Need assessment consists of geriatric assessment measures as criteria to judge eligibility for formal LTC support. Need assessment enables predicting the expenditure from the public LTC system as the number of beneficiaries that can be predicted, and the amount of per capita support (or price) or caps on monthly supports that can also be determined. It also provides an opportunity to "control" public expenditure by controlling the stringency of eligibility and the amount of benefits.

For example, population coverage in LTC insurance is different in different countries. Germany and the Netherlands provide coverage for all types of disability regardless of age. LTC insurance in Japan covers long-term care for older people (65+ years) and age-related LTC for those older people 40–64 years. The Republic of Korea's LTC insurance covers LTC for older people (65+ years) and age-related long-term care of the younger (<65 years). In Japan and the Republic of Korea, stricter eligibility is applied for those below 65 years of age, but they pay contribution, bolstering the financial sustainability of LTC insurance. However, political acceptability by the younger generation is the key because they pay contribution, but their benefits are restricted.

Tax-Based Financing for Long-Term Care

Government can allocate a certain amount of general budget to the LTC sector or designate a specific source of revenue earmarked. Tax (and other government revenues)-based financing is often accompanied by public delivery of LTC, but government can contract with private providers. Alternatively, it can be combined with demand-side financing, such as personal budget, and older people can choose providers that they prefer. One of the key factors in tax financing is the government's political and financial commitment, e.g., willingness of the Ministry of Finance (MOF) to allocate a state budget to LTC financing. Unless government accords political priority to LTC, it can suffer from chronic underfunding.

Tax financing can adopt universal coverage or entitlements in terms of age or need, such as in Scandinavian countries, or target older people in greater need, such as those with low income, high severity and dependency, or those living alone. Alternatively, it can adopt income-related cost sharing, such as high-cost sharing for people with high income and low- or no-cost sharing for those with low income, although it requires administrative capacity to implement means testing or income assessment. In other words, the amount of benefits can be adjusted to the level of income of the recipients even though eligibility is universal.

Public (Mandatory) Insurance for Long-Term Care: Long-Term Care Insurance

Mandatory public LTC insurance (LTCI) has an explicit purchasing mechanism and the insurer contracts with public and private LTC providers. A potential advantage of public LTCI is that the insurance agency can effectively purchase services from providers and improve quality and responsiveness of LTC for older people. Therefore, the capacity and accountability of the purchaser, i.e., LTCI agency, is crucial for the performance of LTCI. In general, LTCI eligibility is based on need, not income, and determined by a formal assessment of LTC need, such as physical and cognitive functions. Even in the mandatory LTCI system, the role of tax or government budget can be important as in the case of Japan, where budget accounts for half of total public funding for LTC; and the Republic of Korea, where government provides subsidy for the contribution of poor people who cannot afford to pay. Nearly all public insurance includes government budgetary-subsidized membership or coverage for poor people and other vulnerable populations.

Earmarking of insurance contribution for LTC can be attractive for resource generation. One of the major challenges of LTCI is how to persuade people to pay contribution, not to mention the difficulty of collecting contribution from the informal sector. Unless contribution is mandatory, it cannot be based on the capacity to pay, i.e., income-based contribution, but enforcement of the mandatory contribution faces political barriers in many low- and middle-income countries. As older people have very limited capacity to pay, it is mainly younger population who should contribute to LTCI. Unlike health insurance, however, the benefits of LTCI are much more concentrated on older people. The need for long-term care due to aging seems remote for many young people and, consequently, their willingness to pay for LTCI tends to be lower than that for health insurance. As a result of the intergenerational redistribution issues, the role of the government budget in LTC insurance is higher than that in health insurance.

GOVERNANCE

Centralization Versus Decentralization

For efficiency of risk pooling and equity of benefits, centralized financing with a single big pool at the national level is usually preferred to decentralized financing with multiple fragmented pools. Compared with health care, however, LTC providers are more labor-intensive, smaller in size, and more decentralized, with a significant role of municipalities or lower levels of governments. Local governments also play a key role in the coordination of LTC and local social services, e.g., transportation and community welfare services. Centralization or decentralization of the LTC system depends on history and sociopolitical culture. For example, municipalities are insurers of the LTC system in Japan (subject to national standards and policy) while LTC in the Republic of Korea is based on a single insurance agency nationwide.

Interministerial Coordination

Coordination between health care (HC) and LTC is a key issue of governance. The boundary between HC and LTC depends on policy, culture, and other factors, and can be arranged in different ways in different countries. In principle, the most important consideration in policy design should be the health and quality of life of older people, for which person-centeredness and the coordination between HC and LTC are crucial. In Japan and the Republic of Korea, health insurance and LTC insurance have separate funding pools, but are under a single ministry. In contrast, most low- and middle-income countries (LMICs) in Asia have two separate ministries of Health and Social Welfare, making interministerial coordination often very difficult and a challenge for the LTC system and policy development in LMICs. When LTC is the responsibility of the Ministry of Social Welfare, it tends to be mainly concerned with LTC for targeted coverage for poor people, older people and people with disabilities rather than universal coverage. As a result, health ministries may be more accountable if the interministerial coordination does not happen.

Relation Between Long-Term Care Financing and Health care Financing

A policy similarity exists between health care financing and LTC financing as a result of path dependency (e.g., political ideology or welfare state orientation) or intention to save administrative costs. When health care is financed by mandatory public insurance, LTC is also likely to be financed by public insurance, as in the case of Germany, Japan, the Republic of Korea, and the Netherlands. In Germany, sickness funds are the insurers of both health care and LTC; local governments are the insurers of both health care for the self-employed and LTC for all participants of LTCI in Japan; and the National Health Insurance Service (NHIS) in the Republic of Korea is the insurer of both health care and LTC. Countries with tax-based financing for health care, such as Australia, Denmark, and the United Kingdom, also have tax-based financing for LTC. However, differences exist in the institutional details of HC and LTC financing in terms of population coverage and eligibility, generosity of benefits, and others.

As tax financing usually relies on general taxation rather than earmarked taxation, separation of HC financing and LTC financing is not an issue in tax-based financing, instead, the relative budget allocation between HC and LTC sector is a concern. In LTC insurance, an important policy decision is whether to extend an existing benefits coverage of health insurance to LTC or to introduce a separate LTC insurance. An advantage of LTCI being separate from health insurance is the potential to reduce the dominant role of the medical profession in LTC provision. However, the separation of health insurance and LTCI may result in difficulty in coordination between health insurance and LTCI, or the lack of continuum of care from health care to LTC for older people.

DISCUSSION

Facing rapid aging and the increase in the demand for LTC, while the availability and willingness of family (informal) caregiving is diminishing, developing member countries (DMCs) in Asia need to formulate national policy and invest in the LTC system. In addition to improving quality of life and health of older people, the LTC system will improve efficiency by reducing social admissions in acute care hospitals and helping family care givers back to labor force. The active role of the private sector in the provision of LTC, as well as increasing employment of certified care workers will contribute to the care economy.

The LTC system should be based on the principle of person-centeredness and continuum of care, ranging from home-based care, day care centers, respite care, to residential care. The LTC system can help older people and family choose different types and levels of LTC following their need and preference without financial barriers. High-income economies have experienced many innovations in residential care, which are well connected to health care and home-based care with an active role of the private sector, leading to aging in place and a community-based care system.

For the care economy, the role of formal care workers is critical. DMCs need to consider their own context and introduce a certificate or licensing system to train care workers. Family caregivers and voluntary workers can provide LTC, but they cannot be the major workforce that LTC can rely on. Therefore, formalizing care workers through standardized training and certification is essential. It is worthwhile to promote regional cooperation in the standardized training and certification of care workers toward mutual recognition of qualification of care workers.

Financing LTC will also depend on the country context, especially the existing financial mechanism for health. When a country has public insurance for health, it can extend the benefit package of health insurance to cover some essential LTC services, considering person-centered coordination of health care and LTC. LTC policy priority of population and service coverage will be higher for the coverage of severe disabilities or residential care than that

for minor disabilities or home-based care. DMCs can envision a approach circle where cost-effective health care improves the functional capacity of older people and reduces the need for LTC; and the LTC system, through universal health and care coverage, improves the health of older people.

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